

**Centre des opérations d'urgences de sante publique
« COUSP-RDC »**

Situation Report of the 17th Ebola Virus Disease (EVD) Outbreak / DRC
SitRep MVE No. 007/MVB_17/2026

Affected provinces	ITURI, NORTH KIVU, SOUTH KIVU
Affected Health Zones	ITURI: Bambu, Bunia, Kilo, Mongbwalu, Nizi, Nyankunde and Rwampara NORTH KIVU: Butembo, Goma and Katwa SOUTH KIVU: Miti-Murhesa
Reporting date	May 21, 2026
Publication date	May 22, 2026

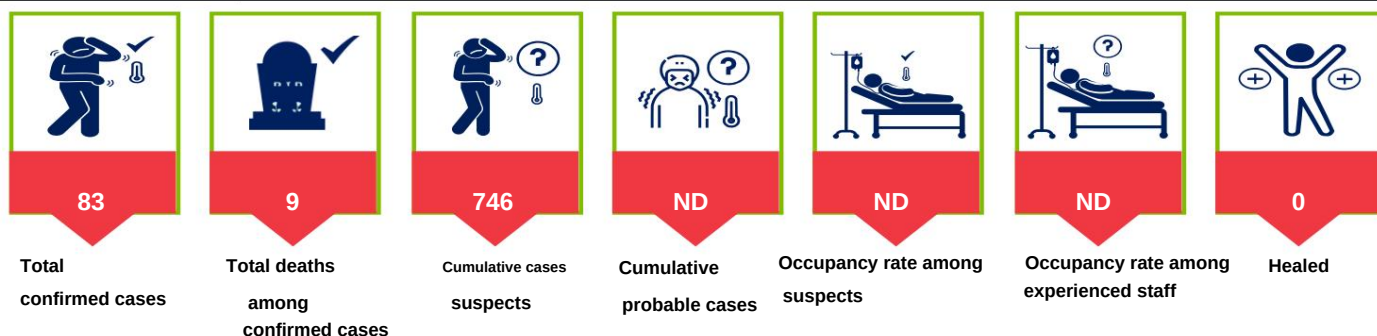


Table I. Distribution of confirmed cases and deaths by health zone in the affected provinces

Affected provinces	Health zones with case confirmed	New case confirmed	New deaths among confirmed cases	New case probable	New case suspects	New contacts listed	Healed by the day
Ituri	Bunia	6	1	ND	41	87	0
	Rwampara	7	1	ND	14	0	0
	Mongbwalu	1	0	ND	5	0	0
	Bamboo	0	0	ND	0	48	0
	Nyankunde	2	0	ND	9	207	0
	Nizi	1	0	ND	1	ND	0
	Kilo	1	0	ND	1	ND	0
Subtotal		18	2	0	70	342	0
North Kivu	Goma	0	0	ND	ND	ND	0
	Butembo	0	0	ND	ND	ND	0
	Katwa	0	0	ND	ND	ND	0
South Kivu	Miti Murhesa	1	1	ND	ND	ND	0
Subtotal		1	1	0	0	0	0
Total		19	3	0	70	342	0



Presentation of the preparedness and response plan for the 17th Ebola Virus Disease epidemic on May 21, 2026 at COUSP, with the Director General of the National Institute of Public Health.



Visit of the DG of Africa CDC to Bunia, Province of Ituri (Meeting with the governor and other partners)

I. CONTEXT

Ituri Province is located in the northeast of the Democratic Republic of Congo and shares a long border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. It has an estimated population of over 8 million, including more than one million internally displaced persons.

In September 2018, this province was affected by an Ebola virus disease epidemic that was raging in the neighbouring province of North Kivu.

The Mongbwalu health zone is one of the 36 health zones in the Ituri province.

Considered the starting point of the epidemic, it is located in the Djugu territory, where several armed groups operate. It is situated 70 km from Bunia, the capital of Ituri province, with frequent population movements towards Uganda. This locality is characterized by several construction sites that attract people from other territories within the province and from neighboring provinces.

II. HISTORY OF THE EPIDEMIC

On May 5, 2026, social media reported a surge in deaths of unknown cause, with nearly 50 deaths already recorded in the Mongbwalu health zone. Following confirmation of this alert, the Chief Medical Officer of the Mongbwalu health zone shared the preliminary investigation report on May 9, 2026. On May 11, 2026, provincial health authorities held an emergency meeting and decided to deploy rapid response teams to Mongbwalu and Rwampara, which had reported suspected deaths from viral hemorrhagic fever among patients hospitalized at the Rwampara Evangelical Medical Center, who had been transferred from Mongbwalu.

Investigations conducted on May 12, 2026, simultaneously in the Mongbwalu and Rwampara health zones by the Rapid Intervention Teams of the Ituri Provincial Health Division (DPS), revealed that most of the patients or suspected deaths presented with fever, headaches, vomiting, and intense physical asthenia. The deaths

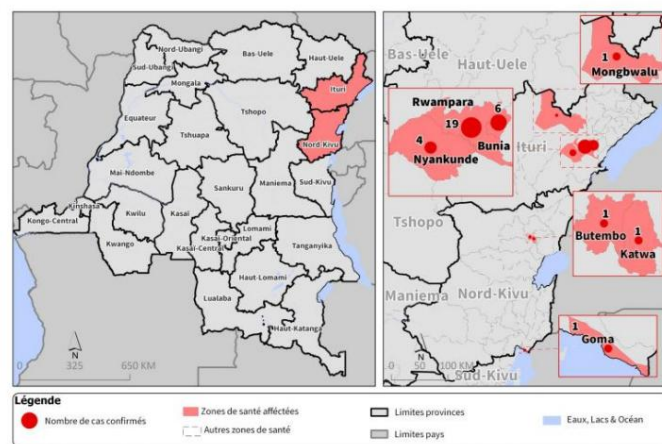
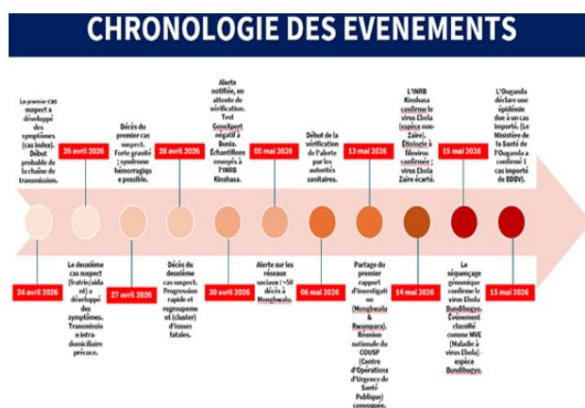
Symptoms appeared 2 to 3 days after the onset of illness, presenting as hematemesis, epistaxis, or blackish vomit. During investigations in the Rwampara health zone, 20 samples were collected from suspected cases or deaths. The majority of samples were taken from the Mongbwalu health zone. Preliminary tests conducted at the Bunia Laboratory were all negative for Ebola Zaire, dengue, rotavirus, cholera, malaria, *Yersinia pestis*, medullary tuberculosis, and COVID-19. These samples were sent to the National Institute of Biomedical Research (INRB) in Kinshasa for further testing.

As of May 14, 2026, **13 of these 20 samples were tested, and 8 were found to be positive** for non-Zairean Ebola virus disease at the INRB in Kinshasa. On May 15, 2026, sequencing of the positive samples identified the BUNDIBUGYO strain, *allowing the Minister of Public Health, Hygiene and Social Welfare (MSHPS) to officially declare the 17th Ebola virus disease outbreak.*

Ebola virus disease (Bundibugyo strain), thus marking the 17th epidemic in the country's history

As of May 17, 2026, in consultation with member states, the WHO Director-General considers that the event taking place in DRC and Uganda meets the criteria of the definition set out in Article 1 of the IHR, and declared that this constitutes a Public Health Emergency of International Concern while specifying that the risk at the global level is low but high at the regional level.

On May 18, 2026, the Executive Director of Africa CDC officially declared a Continental Public Health Emergency (CPHE) under Article 3 of its statutes. This declaration followed an emergency meeting of the Africa CDC Advisory Group, which determined that the worsening outbreak posed an acute risk to continental health security. As a result, the Continental Incident Management Support Team was activated to provide support to affected and at-risk Member States in their response and preparedness efforts.



Les frontières et les autres indications sur cette carte n'impliquent l'expression d'aucune opinion que ce soit de la part de l'Organisation Mondiale de la Santé concernant le statut pandémique de tout pays, territoire, ville ou région sous son autorité, ou concernant la délimitation de ses frontières ou limites. | Source des données: HDPH/WHO, | Réalisation: Equipe HPH Kinshasa | Copyright: OMS 2026

III. EPIDEMIOLOGICAL SITUATION UPDATE

- 19 new confirmed cases, including 18 in Ituri (Rwampara: 7, Bunia: 6, Nyakunde: 2, Nizi: 1, Mongbwalu: 1 and Kilo Mission: 1) and 1 in South Kivu (Miti Murhesa) • The cumulative number of confirmed cases is 83, including 78 in the Ituri Provincial Health Division, 4 in North Kivu, and 1 in the South Kivu.
- A new province affected (South Kivu) through the Miti-Murhesa health zone and a new health zone affected (Kilo Mission) in Ituri province. • Overall, 11 health zones are affected and spread across 3 provinces, including 7 health zones (Ituri), 3 health zones (North Kivu) and 1 ZS (South Kivu).
- The test positivity rate is 45.9% (17/37 samples analyzed).
- The proportion of contacts followed is 21.3% for the day of May 21, 2026.
- Coordination meeting with the Monitoring pillar (refocusing on the pillar's organization monitoring)

IV. RESPONSE ACTIONS

COORDINATION

- A coordination meeting was held, chaired by the Deputy Incident Manager, Head of Division Provincial Health Directorate of North Kivu (Beni) and Ituri (Bunia). • Holding of the partner mobilization meeting on alignment with the response plan.
- Development of the provincial action plan for the Ebola Virus Disease incident in the Ituri Provincial Health Division and North Kivu.

EPIDEMIOLOGICAL SURVEILLANCE

- Drafting of the Terms of Reference for the deployment of provincial teams multidisciplinary in the Mongbwalu health zone.
- Drafting of the Terms of Reference for training stakeholders (ECZS, IT&ITA, community agent leads and data managers) from five active ZSS
- Provision of thermal flash devices in the health zones of Bunia (12) and Rwampara (11) for contact tracing.

Table II. Summary of the alert status as of May 21, 2026

Alert tracking	Alerts postponed	New alerts (20/05)	Total of the alerts of the day (21/05)	Live alerts	Alerts deceaseds	Alerts investedfords	Alerts not investedfords	Alerts validateds	Alerts invalidées	Case suscep all sampled	Case suscep no pre-sampled	Case suscep isolatedts	Case suscep all non-isolated
Bunia	4	39	43	40	3	41	2	41	0	17	24	41	0
Rwampara	6	20	26	19	7	14	12	14	0	12	2	12	0
Nyankunde	0	10	10	9	1	9	0	10	0	10	0	9	0
Mongbwalu	0	7	7	3	4	5	2	5	0	4	1	1	0
Bamboo	0	0	0	0	0	0	0	0	0	0	0	0	0
Nizi	0	1	1	0	1	1	0	1	0	1	0	O	0
Lita	0	1	1	0	1	1	0	1	0	1	0	O	0
Fataki	ND ND	ND ND	ND ND ND	ND ND						ND ND	ND ND		ND
Logo	ND ND	ND ND	ND ND ND	ND ND						ND ND	ND ND		ND
Aungba	ND ND	ND ND	ND ND ND	ND ND						ND ND	ND ND		ND
Rethy	ND ND	ND ND	ND ND ND	ND ND						ND ND	ND ND		ND
Komanda	ND ND	ND ND	ND ND ND	ND ND						ND ND	ND ND		ND
Nyaramba	0	1	1	1	0	1	0	1	0	1	0	1	0
Aru	0	3	3	3	0	3	0	3	0	3	0	3	0
Drodro	0	1	1	1	0	1	0	1	0	1	0	1	0
Mambassa	0	1	1	1	0	1	0	1	0	1	0	1	0
Mangala	ND ND	ND ND	ND ND ND	ND ND						ND ND	ND ND		ND
Lolwa	ND ND	ND ND	ND ND ND	ND ND						ND ND	ND ND		ND

Table III. Summary of contact identification and follow-up as of May 21, 2026

Areas of health	Daily monitoring						tracking	Total	Reason for not being seen						
	Postponed from	May	day	Contacts	Releases	They			Simple	Lost	hours)	Never	Refusal	Displaced	
	New	Contact											Reco	the	
Bunia	203	87	290	87	0	0	% 203		0	0	0	0	0	203	290
Rwampara	621	0	621	0	0	0	0%	621	0	0	0	0	0	621	621
Nyankunde	133	207	340	207	0	0	61% 133		0	0	0	0	0	133	340
Mongbwalu	176	0	176	0	0	0	0% 176		0	0	0	0	0	176	176
Bamboo	128	48	176	48	0	1	% 128		0	0	0	0	0	128	176
Total	1261	342	1603	342	0	1	% 1261		0	0	0	0	0	1261	1603

Table IV. Distribution of suspected and confirmed cases, deaths of Ebola virus disease in the health zones of the Ituri and North Kivu Provincial Health Directorates as of May 21, 2026*

Provinces	Health zones	Number of cases suspects	Number of probable deaths**	Number of cases Confirmed	Number of contacts
Ituri	Mongbwalu	322	84	14	176
	Rwampara	193	51	31	621
	Bunia	166	34	15	290
	Nyankunde	38	6	11	340
	BAMBU	2	0	1	176
	Nizi	3	1	1	ND
	Kilo Mission	1	0	1	ND
	Samples without a datasheet	0	0	4	0
North Kivu	Goma	0	0	1	ND
	Katwa	13	0	2	ND
	Butembo	7	0		ND
South Kivu	Miti Murhesa	1	0	11	ND
Total		746	176	83	1603

*This data is subject to change after harmonization of data at different levels of the health pyramid with the incident management system.

**The classification will be confirmed after further investigation.

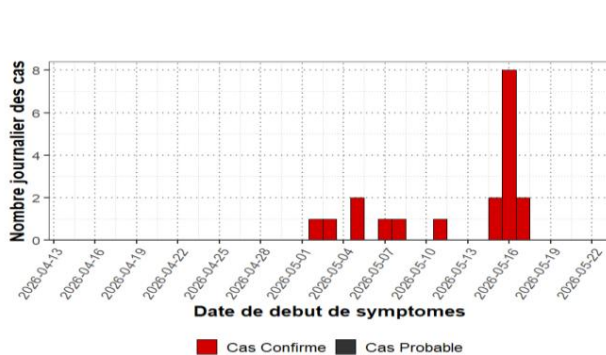
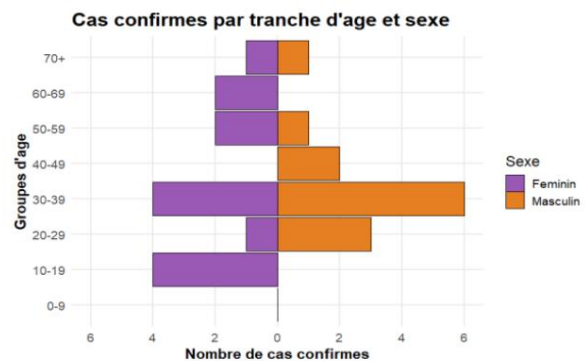


Figure 1. Evolution of confirmed cases in the Ituri DPS by date of symptom onset (source: DHIS2 Partial Tracker)



Point of Entry and Point of Control (PoE/PoC) Surveillance

- Daily meeting to coordinate activities.
- Encoding of PoE/PoC data (database).
- Daily data reporting.
- Supervision and monitoring of activities.

Table V. Data Summary (PoE/PoC) as of 21/05/2026

Health Zone	PoE/PoC	Tot v past	Tot v screenés	Total hand washing	Tot v sensibilisés	All alerts	Total refusal Handwashing	Total refusal screening
NIZI	NIZI BRIDGE	0	0	0	0	0	0	0
BUNIA	AIRPORT	0	0	0	0	0	0	0
	MUDZIBALA	1473	1473	1473	1473	0	0	0
	BRIDGE SIMBILIABO	0	0	0	0	0	0	0
RWAMPARA	SHARI BRIDGE	0	0	0	0	0	0	0
	DELE	940	940	940	940	0	0	0
	CHAI	0	0	0	0	0	0	0
TCHOMIA	TCHOMIA	71	71	71	71	0	0	0
	NYAMUSASI	0	0	0	0	0	0	0
	KASENYI	59	59	59	59	0	0	0
	MANDJE	0	0	0	0	0	0	0
NYAKUNDE	MARABO	0	0	0	0	0	0	0
	NYAK ENTRANCE.	0	0	0	0	0	0	0
	SOTA	0	0	0	0	0	0	0
	BRIDGE TALOLO	0	0	0	0	0	0	0
MUNGWALU	POIPO	0	0	0	0	0	0	0
	MAHIDEDE	0	0	0	0	0	0	0
	PLITO BRIDGE	0	0	0	0	0	0	0
	BRIDGE KIRIKOU	0	0	0	0	0	0	0

LABORATORY

	Total
Total samples received	290
Re-sampling (control)	3
Total samples analyzed	171
Cumulative positives	77

DISTRIBUTION OF SAMPLES TESTED AT THE BUNIA PAR NATURE LAB

Date	Provenance /ZS	Nombre échantillons testés	Natures d'échantillon		Reprélevement	Positifs		Négatifs		Indéterminés	
			Sang	Swab		Sang	Swab	Sang	Swab	Sang	Swab
21/05/2026	Bunia	11	10	1	0	5	1	5	0	0	0
	Rwampara	8	7	1	0	6	1	1	0	0	0
	Nyakunde	15	15	0	0	2	0	13	0	0	0
	Mongbwala	1	1	0	0	1	0	0	0	0	0
	Bambu-mine	1	1	0	0	0	0	1	0	0	0
	Nizi	1	1	0	0	1	0	0	0	0	0
TOTAL		37	35	2	0	15	2	20	0	0	0

DISTRIBUTION OF SAMPLES TAKEN

Date	Provenance /ZS	Reprélevement	Diagnostic				Suivi			
			Résultats P1		Résultats P2		Résultats P3 à P5			
			Positifs	Négatifs	Positifs	Négatifs	Positifs	Négatifs	Positifs	Négatifs
21/05/2026	Bunia	0	0	0	0	0	0	0	0	0
	Rwampara	0	0	0	0	0	0	0	0	0
	Nyakunde	0	0	0	0	0	0	0	0	0
	Mongbwala	0	0	0	0	0	0	0	0	0
	Bambu-mine	0	0	0	0	0	0	0	0	0
	Nizi	0	0	0	0	0	0	0	0	0
TOTAL		0	0	0	0	0	0	0	0	0

TODAY'S RESULTS: 21/05/2026

Date	Provenance /ZS	Nombre échantillons testés	Natures d'échantillon		Reprélevement	Positifs		Négatifs		Indéterminés	
			Sang	Swab		Sang	Swab	Sang	Swab	Sang	Swab
21/05/2026	Bunia	11	10	1	0	5	1	5	0	0	0
	Rwampara	8	7	1	0	6	1	1	0	0	0
	Nyakunde	15	15	0	0	2	0	13	0	0	0
	Mongbwala	1	1	0	0	1	0	0	0	0	0
	Bambu-mine	1	1	0	0	0	0	1	0	0	0
	Nizi	1	1	0	0	1	0	0	0	0	0
TOTAL		37	35	2	0	15	2	20	0	0	0

RISK COMMUNICATION AND COMMUNITY ENGAGEMENT (RCCE)

- Plea to His Excellency the Bishop of the Diocese of Bunia, who was on mission and represented by the Vicar General to obtain his commitment in support of the a
- Advocacy efforts with the mayors of the municipalities of Shari, Nyakasanza, and Mbunya to secure their commitment to supporting communication activities related to the Ebola response
- Awareness-raising of 38 livestock farmers and wholesale traders at the Bunia industrial slaughterhouse on preventive measures against Ebola virus disease (EVD), including 16 women and 22 men
- 9 leaders of the Church of Christ in Congo were made aware of preventive measures against Ebola and agreed to let the CREC team come and deliver the messages during Sunday worship.
- 2 leaders of the Islamic community were sensitized and agreed to the CREC to visit the 5 mosques this Friday, May 22, 2026 (City, City, YAMBI, PRISON and KANYASI).
- 1 leader of the Kimbanguist community is aware and agrees that the CREC will pass on the messages during their Sunday worship service;
- The CHRISCO Church invites the CREC team to share the messages during their Sunday service; • ZS RWAMPARA, 25 people sensitized on preventive measures, including 15 women and 10 men in AS SHARI during the search and follow-up of escaped cases at the Bunia General Referral Hospital

INFECTION PREVENTION AND CONTROL

ZS Bunia

- 2 EDS completed and 1 in progress.
- Briefing of 5 emergency room healthcare providers from HGR-Bunia on preventive measures
- Provision of PPE and decontamination equipment for the practical session of the ZS teams Bunia.

• Decontamination of the isolation building at HGR-Bunia.

• Provision of some PCI inputs by the ALIMA partner

ZS Rwampara

• Briefing of the 4 Balidja Morgue staff members on the wearing of PPE before handling any body ;

• Continued briefing of the 63 healthcare providers (Hygienists, nurses, Doctors and Midwives of CME Nyakunde) on the management of biomedical waste.

• 3 EDS completed and 1 in progress.

• Decontamination of the isolation room at HGR/Rwampara.

• Briefing of 10 surface technicians and 15 PCI supervisors on the port and removal of PPE.

ZS Mongbwalu

• Formative supervision in 4 health facilities including: Uponyaji Health Center, Rwampara Health Center, Post of Amani health and CS Tchungu (without doing the evaluation);

DPS North Kivu

- Decontamination of Matanda Hospital where the confirmed case from the Katwa health zone is staying;
- Matanda Hospital PCI scorecard evaluation: 70.6%; • Case mapping completed;
- Status of the CTE for the release of PCI input needs (PPE, water supply, waste management, patient circuit).

INFECTION PREVENTION AND CONTROL

- Development of the logistics needs report for ZS Goma, Katwa, Beni, Karisimbi, Mutwanga, Vuhovi, Kalunguta, Kamango, Mabalako, Oicha, Museniene, Kayina, Alimbongo, Butembo;
- Acquisition of Starling kits for internet connection from UNICEF;
- Receipt of 5 vehicles provided by the WHO;
- Receipt of 200 litres of diesel (Support from MEMISA-Belgium).

HOLISTIC CARE

- Briefing of service providers at HGR Bunia and Rwampara on the wearing of PPE;
- Clinical evaluation of patients;
- Delivery of 5 mattresses, 5 beds and other equipment to the CME
- Two isolation tents are being set up at the Bunia General Referral Hospital;
- Fitting out of a room at the Bunia General Referral Hospital is underway.
- Evaluation of the nutritional status of patients in isolation at the CME.
- 14 announcements of results;
- Awareness-raising of 97 family caregivers, 9 hygiene specialists, 11 workers, 92 teachers from CSJM and 25 taxi drivers;
- Family interviews with 4 families of deceased;

Table VI. Patient movement in healthcare facilities as of [date]
May 21, 2026

Indicators		HGR Bunia	RWAMPARA			Mongbwalu	Nyankunde Grand	Total
Report		36	21	15	35	27	16	129
News Admissions	PPL	1	0	0	0	0	7	8
	Others	13	2	4	6	9	0	28
	Total	14	2	4	6	9	7	36
Total Isolation		50	23	19	46	36	23	165
Outings	Healed	0	0	0	0	0	0	0
	Deceased	8	4	4	10	5	0	23
	Cumulative deaths	46	10	15	25	44	1	116
	No Case	0	0	0	0	0	0	0
	Escapees	9	0	0	0	1	0	10
	Transferred to HGR	0	0	0	0	0	0	0
Patients in isolation	Suspects	48	21	17	38	35	17	138
	of which were taken	26	ND	1	1	0	7	34
	Including severe/critical ones	12	1	1	2	0	0	14
	Including pregnant women	0	0	0	0	1	0	1
	Including breastfeeding women	2	0	0	0	1	0	3
	Including Suspect Child < 5 years	6	0	0	0	1	0	7
	5 to 14 years old	3	0	0	0	2	0	5
	Total confirmed cases	4	2	2	4	1	6	15
	Including new cases Confirmed	2	0	0	0	1	1	4
	Including AC Confirmed	0	2	1	5	0	4	9
	Including severe/critical ones	2	0	1	2	0	0	4
	Including those with comorbidities	1	0	0	0	0	0	1
	Including pregnant women	0	0	0	0	0	0	0
	Including breastfeeding women	0	0	0	0	0	0	0
	Including Suspect Child < 5 years old	0	0	0	0	0	0	0
	5 to 14 years old	0	0	0	0	0	0	0

SECURITY

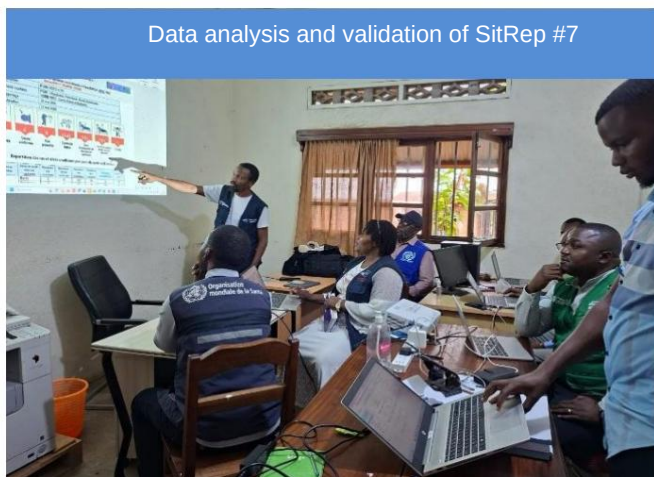
- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm
- Security briefing for deployed teams.

V. CHALLENGES

- Lack of standardized CT and CTE facilities
- Shortages of IPC equipment (PPE, DLM, chlorine) in North Kivu
- Poor reporting of alerts from the zones;
- Insufficient financial resources to support all pillars of the response;
- Difficulty transporting samples to laboratories in Goma and Kinshasa.
- Spread of rumors and proliferation of traditional treatment methods.
- Lack of communication materials and tools.
- Low diagnostic capacity
- Insufficient quantity of inputs and reagents

VI. Recommendation

- Make the CT and CTE facilities in Beni, Butembo, and Goma operational (rehabilitation and circuit review)
- Strengthen surveillance to facilitate the reporting of alerts and the tracing of contacts who have never been seen and lost to follow-up;
- Mobilize resources to support the various pillars of the response;
- Train service providers on Ebola and Sjögren's syndrome surveillance in affected and high-risk health zones risk ;
- Training of Community Health Workers on Ebola and SBC Surveillance in Affected Health Zones and high risk.



Receipt of 14 tonnes of logistical equipment, PPE and PEC kits from Kinshasa (WHO) and 19 tonnes of inputs and IPC kits (UNICEF)



Fire at the Rwampara isolation center



Installation of a handwashing station at the PoC

**FOR ALL
INFORMATION
ADDITIONAL,
PLEASE CONTACT**

For the National Institute of Public Health (INSP) of the DRC

The Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

E-mail: diedonne.mwamba@insp.cd Tel. :
+243 816 040 145

The COU-SP Coordinator

Professor NGANDU Christian

E-mail: Christian.ngandu@insp.cd Tel. :
+243998091915

The Incident Manager

Professor Pierre AKILIMALI

E-mail: pierre.akilimali@insp.cd Tel. :
+243815800288

For the World Health Organization (WHO/DRC)

The WHO Representative in the DRC

Dr Anne Marie ANCIA

Email: anciaa @who.int

The WHO EPR Cluster Team Lead in the DRC, Dr.

FOTSING Richard, Email:

fotsingri@who.int

For Africa CDC

Justus Nsio Mbete E-

mail: JustusNs@africacdc.org